SLCT — SURGEON LEARNING CURVE TRACKING

SIMULATED · v0.4

Study / Protocol	Site / Code	SLCT Record ID / Revision	Record Status
GLENOS-01 / GS-2026-01	Riverside Orthopedic Center / 014	SLCT-014-S03-2026 / r1	Active / periodic review
Surgeon Code S03	Procedure Family / Anatomical Field Primary reverse arthroplasty / shoulder	Device / System GLENOS shoulder system v2	Date Opened 29 Sep 2026
Current SCR / CCDM / TRE Reference SCR-014-03 r1; CCDM-014 v0.2 / T03; TRE-014-03 r1		Applicable CIP / Procedure / IFU Version CIP v4.2 / technique v2 / IFU v3.1	**Review type** ☐ Initial ☐ Early stabilization ☒ Periodic ☐ Triggered

A Purpose, Decision and Boundary

This log follows the procedural stabilization of one **surgeon** performing one defined **procedure family** with one defined **device/system**. It records context-sensitive signals close to the operative act so that surgeon-dependent variation can be reviewed before it appears as a later safety, deviation or data-quality problem.

Decision produced: Is execution within the defined scope becoming and remaining sufficiently stable for the current case-complexity context, or is intensified review, retraining, proctoring, restriction or formal escalation required?

This is not an entry credentialing record, a device-training record, an adverse-event form, a device-deficiency form or an intraoperative source record. It receives selected, traceable information from those records and documents the learning-curve interpretation and resulting control decision. Use the surgeon code in this log; the PI keeps the identity key separately.

B Comparable-Case and Complexity Framework

Complete this framework when the log is activated, before case entry. It is the fixed comparison key for the surgeon, procedure family and device/system. Update it only when the procedure, device, indication, technique, complexity definition or applicable control changes. Each case record then refers to this framework rather than redefining the comparison in every line.

Comparison element	Local definition or rule to apply	Evidence source / document reference	Effective date / update
Procedure / device scope	Primary reverse arthroplasty, GLENOS v2. No revision or grafting cases.	CIP v4.2 / procedure IFU v3.1 SCR-014-03 D1	29 Sep 2026
Case-complexity strata	Lower: preserved glenoid. Standard: routine cuff-tear arthropathy. Higher: augmentation/revision, excluded.	Procedure plan PP-014 v1 Specialty criteria SC-014 v1 SRR-014 / R03	29 Sep 2026
Operative-duration comparator	Lower: 80–110 min. Standard: 100–130 min. Planning ranges, not proficiency cutoffs.	Local plan PP-014 v1 Comparable-case basis, Sec. 3	29 Sep 2026
Expected initial support	Dr L. Costa at first five cases. Support changes only through the SCR review.	SCR-014-03 D2 Proctor plan PP-014 v1 CCDM T03 / S1	29 Sep 2026
Review triggers	Two unplanned size changes in comparable cases, unexplained duration drift or a safety signal.	SRR-014 / R03 POT-014 / trigger plan	29 Sep 2026

Enter the supporting document and its version, section or record ID. The source is a controlled document or record, not the name of a reviewer.

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Initial case cohort

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C Case-Level Tracking — Initial Cases 1 to 5

Case seq.	Date / case source reference	Procedure / device / configuration	Complexity class and key factor	Operative duration / comparator	Unplanned substitution or technical deviation	Senior support / proctoring	Documentation / early technical outcome / linked record
1	29 Sep 2026 / ICDL-014-C01	Reverse / GLENOS v2 / GS36	Standard / routine anatomy	128 min / 100–130	None observed	LC present as planned	Record complete; no early concern. IMG-014-C01.
2	01 Oct 2026 / ICDL-014-C02	Reverse / GLENOS v2 / GS36	Lower / preserved glenoid	110 min / 80–110	None observed	LC present as planned	Record complete; no early concern. IMG-014-C02.
3	02 Oct 2026 / ICDL-014-C03	Reverse / GLENOS v2 / GS36	Standard / routine anatomy	123 min / 100–130	None observed	LC present as planned	Record complete; no early concern. IMG-014-C03.
4	05 Oct 2026 / ICDL-014-C04	Reverse / GLENOS v2 / GS36	Lower / preserved glenoid	102 min / 80–110	None observed	LC present as planned	Record complete; no early concern. IMG-014-C04.
5	08 Oct 2026 / ICDL-014-C05	Reverse / GLENOS v2 / GS36	Standard / routine anatomy	118 min / 100–130	None observed	LC present as planned	Record complete; no early concern. IMG-014-C05.

Record a concise source reference, not participant-identifying information. State “none observed” where an expected variation, deviation or support event did not occur; do not leave a negative finding ambiguous.

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Cohort and scheduled review

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D Initial-Cohort Review

Review the first cohort against the comparison framework. Interpret patterns across comparable cases rather than any isolated event, and record a linked action whenever a concern or material limitation is identified. Compare with this surgeon's earlier cases of similar complexity and distinguish planned from additional support.

Review question	Evidence and context assessed	Conclusion	Required action / linked record
Are operative duration and support needs interpretable within comparable complexity strata?	09 Oct / LC: standard 128, 123, 118 min; lower 110, 102. Planned support in all five.	☒ Stable ☐ Observation ☐ Concern	No added control. SCR-014-03 G reviewed.
Are substitutions or technical deviations isolated, expected or showing recurrence?	09 Oct / LC: no unplanned substitution or technical deviation in cases 1–5.	☒ Isolated / expected ☐ Observation ☐ Concern	Continue within scope; review 6–10.
Do documentation, imaging or early technical findings suggest a procedural pattern?	09 Oct / LC: operative/image records complete; no early technical pattern.	☒ No pattern ☐ Observation ☐ Concern	Routine capture; no safety handoff needed.

E Scheduled Review Register

Use one row for each planned cohort or periodic review. Summarize the complexity range, the observed pattern, the decision made and the next review or linked control record. Include the actual review date and reviewer in each entry.

Review point / period	Cases and complexity context reviewed	Pattern across duration, variation, support and evidence quality	Decision / next review / linked records
Cases 1–5	09 Oct 2026 / LC. Cases 1–5: three standard, two lower.	Duration within local ranges; planned support; no technical pattern.	Continue; SCR-014-03 G confirms end criteria of temporary controls. Review 6–10.
Cases 6–10	29 Oct 2026 / LC. Cases 6–10: four standard, one lower.	Two size changes interpreted with anatomy; G01 completed. No unexplained drift.	Continue in reviewed scope. Next cohort 11–15; review by 20 Nov.
Periodic review	30 Oct 2026 / MS + LC. Ten cases to date.	G01 closed; no new signal since case 10. No fixed-count proficiency claim.	Stable for current scope. Next review 20 Nov 2026.
Other defined cohort	Not opened as of 30 Oct 2026.	No additional cohort accrued.	Not due; define if scope or device changes.

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Continuation case cohort

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F Case-Level Tracking — Continuation

Case seq.	Date / case source reference	Procedure / device / configuration	Complexity class and key factor	Operative duration / comparator	Unplanned substitution or technical deviation	Senior support / proctoring	Documentation / early technical outcome / linked record
6	12 Oct 2026 / ICDL-014-C06	Reverse / GLENOS v2 / GS36	Standard / routine anatomy	116 min / 100–130	None observed	No additional support	Record complete; no early concern. IMG-014-C06.
7	15 Oct 2026 / ICDL-014-C07	Reverse / GLENOS v2 / GS36	Lower / preserved glenoid	98 min / 80–110	None observed	No additional support	Record complete; no early concern. IMG-014-C07.
8	19 Oct 2026 / ICDL-014-C08	Reverse / GLENOS v2 / GS40	Standard / wider glenoid	121 min / 100–130	GS36 to GS40 after trial; within IFU. ICDL F.	No additional support	Sizing rationale retained; review G01. IMG-014-C08.
9	22 Oct 2026 / ICDL-014-C09	Reverse / GLENOS v2 / GS40	Standard / eccentric wear	126 min / 100–130	GS36 to GS40 after trial; within IFU. ICDL F.	No additional support	Sizing rationale retained; review G01. IMG-014-C09.
10	26 Oct 2026 / ICDL-014-C10	Reverse / GLENOS v2 / GS36	Standard / routine anatomy	114 min / 100–130	None observed	No additional support	Record complete; no early concern. IMG-014-C10.

Attach continuation pages using the same column order for additional cases. Enter the next case numbers; do not restart at 6 when this sheet is reprinted. Keep a distinct log for a materially different procedure family or device/system unless the approved comparison framework explicitly permits aggregation.

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Triggered assessment and action

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G Triggered Learning-Curve Assessment

Open this assessment when a pre-defined or material signal appears. Triggers may include repeated unplanned component substitutions, duration consistently outside the comparable-case expectation, recurrent technical deviations, clustering of early technical complications, repeated need for senior support, a new device/technique/indication, or an external signal from CSST, SRR, IMC or PI oversight.

Trigger and date	Cases / records reviewed	Complexity-aware assessment	Likely control issue, if any	Immediate disposition
G01 / 23 Oct 2026: two size changes.	Cases 8/9; ICDL F and images; IFU v3.1.	29 Oct / LC: anatomy explains sizing changes; no unexplained drift.	No technical failure found.	Continue; assessment closed 29 Oct.
Not used				
Not used				
Not used				
Not used				

Do not attribute a safety signal to surgeon performance without reviewing complexity, device, procedure, patient and documentation context. A confirmed complication signal should be assessed against this log before it is attributed to the device or to the operator.

H Action, Restriction and Reassessment

Signal / assessment reference	Action or restriction	Owner / due date	Evidence required for reassessment	Outcome / linked record / closure
G01 / cases 8–9	Review anatomy and sizing rationale; no new restriction.	LC / 29 Oct 2026	ICDL F, images and IFU sizing rules.	Completed 29 Oct; PP-014 review note. Continue surveillance.
Not used				
Not used				
Not used				
Not used				

A restriction, training intervention or intensified review is not closed merely because it was initiated. Record the reassessment evidence and whether the control was effective, insufficient or still under review.

I Disposition Logic

Use this logic only after completing the scheduled review or a triggered assessment. It converts the conclusion into a controlled response. The selected outcome is recorded in the action log and in the overall status below; it is not a second assessment or a substitute for the trigger table.

Disposition	Meaning	Required destination or response
Continue as planned	The available evidence is stable within the approved scope and comparable case-complexity context. No material surgeon-dependent drift is identified.	Record next scheduled review and retain source references.
Intensify review	A pattern is uncertain, early or limited. It does not yet justify a scope restriction but requires closer observation or a shorter review interval.	Define additional cases, reviewer and next review date.
Adapt controls	Evidence supports a local intervention such as renewed device training, temporary proctoring, co-surgery, enhanced documentation review or restriction to lower-complexity cases.	Update TRE, SCR or CCDM as applicable; open a dated action and reassessment point.
Formal escalation	A material, recurrent or unresolved pattern requires risk, governance, safety or sponsor review according to the applicable procedure.	Open or update SRR / CAPA; notify PI oversight, CSST or sponsor route as applicable.

J Overall Record Status and Certification

I confirm that this log was reviewed against the applicable source records for the surgeon, procedure family and device/system stated in the header. The conclusion reflects complexity-aware interpretation of the recorded data. It does not replace safety assessment, device deficiency assessment, participant-level clinical judgment or the PI's retained responsibility for the clinical investigation.

Current learning-curve status	Select one and state the current scope or condition	Effective date / next review
Stable / continue	☒ Current procedural scope and supervision remain appropriate.	30 Oct 2026 Next: 20 Nov 2026
Under intensified review	☐ Additional cases or a shorter review interval are required.	
Control adaptation active	☐ Training, proctoring, scope or documentation control has been changed.	
Escalated / restricted	☐ Formal risk, governance, safety or sponsor route has been activated.	

Primary reviewer

Maria Silva / coordinator
M. Silva
30 Oct 2026

Surgical / clinical reviewer

Dr Luis Costa / surgeon
L. Costa
30 Oct 2026

Principal Investigator / accountable approver

Dr Alex Morgan / PI
A. Morgan
30 Oct 2026

Next scheduled review / controlled record location

20 Nov 2026
Site file / SLCT-014-S03
r1 / v0.4